Atezolizumab + nab-paclitaxel

NCCP 00688 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Atezolizumab 840 mg is given on Days 1 and 15, before nab-paclitaxel 100 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Nab-paclitaxel is planned for up to six cycles; extension needs consultant review. Atezolizumab may continue after chemotherapy stops.

STEP 1

Before treatment

Baseline FBC, kidney/liver function, glucose, thyroid function, hepatitis B/C testing and morning cortisol. FBC, kidney/liver tests and glucose before each cycle; check the count results needed for Day 8 and Day 15 decisions.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Atezolizumab is given first, then nab-paclitaxel; Day 8 has nab-paclitaxel only.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Atezolizumab: 60 minutes initially, then 30 minutes if tolerated; nab-paclitaxel: 30 minutes. Full visits also include preparation, review and observation.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Separate combination, Day 8 chemotherapy-only and immunotherapy-only support; maintain immune and blood-count monitoring.

Sources: NCCP 00688 v2a · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.